

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in California

A licensed Home Care Organization (HCO) — a private-pay, VA & LTC-insurance business.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The California model — read this first

In California, an agency that sends caregivers into people's homes to help with daily living — bathing, dressing, meals, light housekeeping, companionship — must be licensed by the California Department of Social Services (CDSS), Home Care Services Bureau, as a Home Care Organization (HCO) under the Home Care Services Consumer Protection Act (Health & Safety Code § 1796.10 et seq.). This is NON-MEDICAL care. Two things make California distinctive: every caregiver you employ must be registered on the state's Home Care Aide Registry, and — importantly for your business plan — a licensed HCO is overwhelmingly a PRIVATE-PAY business. This guide walks the whole path, broken down so a first-timer can follow it.

The single most important California fact — read before you build a business plan. California's big public program for in-home personal care is IHSS (In-Home Supportive Services), which serves roughly 900,000 people. But IHSS pays INDIVIDUAL providers directly — the client hires their own caregiver (often a family member) and the state pays that person by timesheet. IHSS largely BYPASSES agencies; a licensed HCO can only touch it through “contract mode” in the minority of counties that offer it. So a California HCO's revenue is mostly PRIVATE PAY (plus long-term-care insurance, VA, and — for the developmental-disability population — regional-center contracts). Build your model around private pay from day one (Part 8).

Accreditation — optional here, but it opens doors. California does NOT require national accreditation to hold an HCO license. But accreditation (ACHC, CHAP, or The Joint Commission) is often required by VA contracts, long-term-care insurers, and franchise brands, and helps you win referral partners. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (see Part 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with CDSS/HCSB and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, operating, paid agency. Each part below is one rung.

1	Decide this is your business	A licensed non-medical home-care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, and the required insurance & bond (Part 3)
3	Get your HCO license	CDSS application, the fee, bond & insurance proof (Part 4)
4	Register your caregivers	The Home Care Aide Registry, Live Scan, TB, training (Part 5)
5	Set up operations	Policies, aide training compliance,

		unannounced-visit readiness (Part 6)
6	Serve private-pay clients	Your core market — start earning on your license (Part 8)
7	Add VA & LTC-insurance	VA Community Care Network + long-term-care-insurance clients (Part 8)
8	Consider regional-center work	Vendorize for developmental-disability clients (Part 8)
9	Get your first clients	Discharge planners, home-health/hospice partners, AAAs (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Two things gate you: the HCO license (with its bond and insurance) and getting your caregivers onto the Home Care Aide Registry (Live Scan + TB + training) before they see a client. Start both early, budget the \$800 California franchise tax, and plan your business around private-pay clients.

PART 1

What This Business Is

In short: A licensed Home Care Organization — caregivers (registered Home Care Aides) who help clients with daily living in their own homes. Non-medical, agency-based, mostly private pay.

California law (HSC § 1796.12) defines a Home Care Organization as an entity — including an LLC — that arranges for home care services: non-medical assistance provided to a client who, because of advanced age or a physical or mental disability, needs help with the activities of daily living (bathing, dressing, grooming, eating, toileting, mobility) and household tasks (meals, light housekeeping, errands, companionship). Your caregivers are “home care aides,” and each must be listed on the state registry.

HCO (non-medical) vs. a Home Health Agency (skilled)

	Home Care Organization (HCO)	Home Health Agency (skilled)
What it is	Non-medical help with daily living	Nursing, therapy, wound care
Who delivers it	Registered Home Care Aides	RNs, LVNs, PT/OT/ST
Who licenses it	CDSS — Home Care Services Bureau	CA Dept. of Public Health (CDPH)
Your license	Home Care Organization	Home Health Agency (different, harder)

This guide is for the non-medical HCO. Skilled home health is a separate, CDPH-licensed (and Medicare) business you could add later.

The two people the law regulates: the HCO and the Home Care Aide

- **Home Care Organization (HCO):** the licensed agency — you. You hold the license, the bond, and the insurance, and you employ the caregivers.
- **Home Care Aide (HCA):** the individual caregiver, who must be on the Home Care Aide Registry (background-cleared). An “affiliated” aide is your W-2 employee; an “independent” aide works directly for clients. Your caregivers are affiliated aides.

Key terms. CDSS = California Dept. of Social Services (your licensor). HCSB = Home Care Services Bureau. HCO = Home Care Organization (the agency). HCA = Home Care Aide (the caregiver). Registry = the Home Care Aide Registry. IHSS = In-Home Supportive Services (the county-run Medicaid personal-care program that pays individual providers). Live Scan = electronic fingerprinting. LTC insurance = long-term-care insurance. Regional Center = the developmental-disability services system. CCN = the VA's Community Care Network.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. California is the largest home-care market in the country — with roughly 6 million residents aged 65+, some of the highest private-pay rates in the nation, and a population that is aging faster than the state can build facilities to house it. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) *undercapitalization* — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) *weak caregiver recruiting and retention* — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) *sloppy compliance* — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *CDSS/HCSB licenses you and runs the aide registry, the counties run IHSS (which mostly bypasses agencies), the VA buys care through its network, and regional centers fund the developmental-disability population.*

CDSS — Home Care Services Bureau (your licensor)

The California Department of Social Services, Community Care Licensing Division, Home Care Services Bureau, issues your HCO license, maintains the Home Care Aide Registry, processes aide background checks, investigates complaints, and conducts unannounced visits. Phone (877) 424-5778; HCSB@dss.ca.gov; cdss.ca.gov (Home Care Services).

The counties & IHSS (mostly not your channel)

In-Home Supportive Services is administered by the counties and overseen by CDSS. It's the dominant public payer for in-home personal care, but it pays individual providers directly (the client-directed model), so it largely bypasses agencies — an HCO participates only through county “contract mode,” where it exists. Know it as the landscape you operate in, not your main revenue source (Part 8).

The VA and its Community Care Network

The U.S. Department of Veterans Affairs buys non-medical home care for eligible veterans through contracted agencies in its Community Care Network (CCN). California is in CCN Region 4, administered by TriWest — a real agency channel (Part 8).

Regional Centers (for developmental disabilities)

California's 21 regional centers (funded by the Department of Developmental Services) purchase in-home respite, personal assistance, and supported-living services for people with developmental disabilities. An HCO can become a “vendored” provider to serve that population — a genuine non-private-pay channel.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In California you form the LLC at the Secretary of State by filing Articles of Organization (Form LLC-1), then file an initial Statement of Information (Form LLC-12) within 90 days. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization (Form LLC-1).** File online at bizfileOnline.sos.ca.gov (Business Programs, 916-657-5448); the filing fee is \$70. Name the LLC and list a registered agent (can be you).
- 2. File your Statement of Information (Form LLC-12).** \$20, due within 90 days of forming, then every two years thereafter.
- 3. Budget the \$800 annual franchise tax.** California charges every LLC a minimum \$800 annual franchise tax. The old “first-year free” break expired at the end of 2023, so an LLC you form now owes the \$800 for its first tax year — plan for it (Franchise Tax Board).

No newspaper publication in California. *Unlike New York, California does not require you to publish a formation notice — but the \$800 annual franchise tax and the biennial Statement of Information (Form LLC-12) are real recurring obligations.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it if you bill the VA's Community Care Network or a regional center — and to enroll should you ever touch Medi-Cal or IHSS. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a major bank with lots of California branches — Chase, Bank of America, Wells Fargo, US Bank, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A home-care agency sends caregivers into clients' homes, so you need coverage built for that risk — and California spells out in statute (HSC § 1796.37) exactly what an HCO must carry as a condition of licensure, so this isn't optional shopping, it's a checklist:

Coverage	What it protects against	Must-have?
General & professional liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE — California requires at least \$1M per occurrence AND \$3M aggregate	Yes — required to license
Workers' compensation	Caregiver injuries — California requires a valid policy covering your affiliated home care aides	Yes — required to license
Employee dishonesty bond	Theft from a client's home — California requires at least \$10,000 including third-party coverage (CDSS Form HCS 402)	Yes — required to license
Non-owned & hired auto	Your caregivers driving their own cars for or with clients	Strongly advised
Abuse & molestation	Allegations of abuse by a caregiver — general liability usually EXCLUDES this	Strongly advised
Cyber liability	A breach of client health information	Recommended

Three are mandatory, and you file proof to get licensed. The \$1M/\$3M liability policy, workers' comp, and the \$10,000 employee-dishonesty bond aren't just good ideas in California — you submit proof of them with your HCO application. Add non-owned auto and abuse & molestation coverage too; both are core home-care risks a basic policy leaves out.

Real places to get it

Buy through an independent agent or broker who writes home-care programs — they can also place your \$10,000 employee-dishonesty bond. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan, Gallagher, and NIP Group / HomeCareUSA. Find a local California agent at [trustedchoice.com](https://www.trustedchoice.com) (filter for home care), or ask a franchise or peer. Confirm the carrier meets the § 1796.37 limits and writes non-medical home care in California before you rely on a quote.

What to ask the insurance agent

“Which carriers do you write for California Home Care Organizations, and what's the minimum premium?”

“Can you meet the § 1796.37 limits — \$1M per occurrence / \$3M aggregate liability — and place the \$10,000 employee-dishonesty bond (HCS 402)?”

“Is abuse & molestation included or a sub-limit, and do you add non-owned & hired auto (my caregivers drive their own cars)?”

“Can you set up California workers' compensation covering my affiliated aides?”

“What coverage limits do the VA, regional centers, or long-term-care insurers require?”

PART 4

Get Your HCO License — Step by Step

In short: Assemble the CDSS application with your entity documents, bond, insurance proof, policies, and a training plan; pay the fee; and submit to the Home Care Services Bureau.

4.1 Before you apply

- ☐ Entity formed (LLC), EIN obtained
- ☐ The three required coverages bound: \$1M/\$3M liability, workers' comp, and the \$10,000 dishonesty bond
- ☐ Your policies & procedures, job descriptions, and a caregiver TRAINING plan written
- ☐ A plan to register and screen your home care aides (Part 5)

4.2 The application (CDSS Home Care Services Bureau)

- 1. Get the application packet.** Use CDSS Form HCS 200 (Application for Home Care Organization License) and the HCS 281 instructions — from cdss.ca.gov (Home Care Services).
- 2. Complete the required forms.** Including HCS 215 (Licensee Applicant Information), HCS 308 (Designation of Responsibility), HCS 309 (organization structure), HCS 402 (dishonesty bond), LIC 508 (Criminal Record Statement), and HCS 9165 (Board of Directors statement).
- 3. Attach the supporting documents.** Your Articles of Organization, job descriptions, personnel policies, your training plan, an HCO program description, and proof of liability + workers' comp insurance.
- 4. Pay the fee.** The HCO license fee is \$5,603, covering a two-year license period (CDSS now publishes a single two-year fee). Pay by check/money order to "California Department of Social Services."
- 5. Submit to HCSB.** Mail the complete packet to the Home Care Services Bureau; processing doesn't begin until all documents and payment are received.

Owners and office staff get screened too. Any owner, officer, or staff member with client contact must clear the same criminal-background requirements as your aides (Live Scan, Part 5). Do your own Live Scan as part of the application.

4.3 Timeline

CDSS doesn't publish a guaranteed turnaround, so don't promise yourself a date — assemble a complete packet (a missing bond or insurance proof is the most common delay), and run your caregiver registrations (Part 5) in parallel so you're ready to serve clients as soon as the license issues.

PART 5

Register & Train Your Caregivers — the Home Care Aide Registry

In short: Every caregiver you employ must be on the Home Care Aide Registry *BEFORE* they see a client — that means Live Scan, a TB test, and California's required training. This is the distinctive California step.

5.1 The Home Care Aide Registry

California keeps a public registry of background-cleared caregivers. Every affiliated home care aide (your W-2 caregiver) must be registered before providing care.

1. **Submit the aide's registration.** Through CDSS's system, with the \$35 registration fee (valid for two years).
2. **Complete Live Scan fingerprinting.** The aide is fingerprinted via a DOJ-approved Live Scan vendor; the results go to the California DOJ and the FBI (Part 5.3).
3. **Clear the background check.** The aide must receive a criminal-record clearance (or an approved exemption) before providing care — you can track status in the registry.

5.2 TB test & training (HSC §§ 1796.44–.45)

- **TB test:** each affiliated aide is examined for tuberculosis 90 days before, or within 7 days after, employment, and (if negative) re-examined at least every two years; a chest X-ray is required only if the test is positive.
- **Entry-level training — 5 hours before any client contact:** 2 hours of orientation (the aide's role and terms of employment) plus 3 hours of safety training (basic safety, emergency procedures, infection control).
- **Annual training — 5 hours:** population-specific topics — clients' rights and safety; meeting daily-living needs; recognizing, preventing, and reporting abuse and neglect; personal hygiene and other home-care services; and safe transportation (if you transport clients).
- **Training may be online.** Keep a training record for each aide — CDSS checks it on an unannounced visit.

Watch the 2027 sunset. The current home-care-aide training statute (§ 1796.44) is scheduled to sunset January 1, 2027 unless the Legislature reauthorizes it. The 5-hour entry / 5-hour annual structure is current now — confirm the operative requirement when you onboard aides after that date.

5.3 Background checks — Live Scan (DOJ + FBI)

1. **Live Scan everyone required.** Every aide, plus any owner/officer/staff with client contact, is fingerprinted via Live Scan to the California DOJ and FBI.
2. **Get the clearance (or exemption).** No one provides care until they have a criminal-record clearance or a CDSS-granted exemption; some offenses are non-exemptible.
3. **Confirm current fees.** The DOJ fee, the FBI fee, and the Live Scan site's rolling fee change periodically — confirm current amounts with CDSS's Care Provider Management Branch (1-888-422-5669) before you quote a caregiver a figure.

5.4 Hiring & keeping great caregivers

California treats agency caregivers as W-2 employees (not contractors), so you handle payroll, workers' comp, and supervision. Beyond the legal minimums, invest in orientation, skills check-offs, and consistent scheduling — caregiver quality and consistency are the number-one driver of client satisfaction, referrals, and retention in home care.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Policies & Getting Operational

In short: Write your policies, keep your aide registry/training/TB records current, and be ready for CDSS's unannounced visits. Set up your scheduling and billing systems before your first client.

6.1 Your policies & procedures

Build a P&P Manual covering client rights, intake and assessment, the service plan and supervision, caregiver hiring/registry/training compliance, complaint handling, incident and abuse/neglect reporting, emergency preparedness, and infection control. This is your operating backbone and the first thing a CDSS analyst reviews.

6.2 Keep your compliance records current

- ☐ Every aide registered on the Home Care Aide Registry with a current clearance
- ☐ Each aide's TB screening on file and up to date
- ☐ Each aide's 5-hour entry training (before first client) and 5-hour annual training documented
- ☐ Owner/staff background clearances on file
- ☐ Current bond and insurance certificates (\$1M/\$3M liability, workers' comp, \$10K bond)

CDSS conducts unannounced visits, so treat these records as always-inspection-ready. A missing training log or an expired TB test is the most common finding.

6.3 Set up your systems

Choose a home-care software platform for scheduling, caregiver and client records, visit tracking, invoicing, and payroll. Since you're mostly private pay, you won't need Medicaid EVV — but if you later take any Medicaid-funded personal care, electronic visit verification applies, so pick software that can support it.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (usually privately), you assess and set up the service plan, a registered aide delivers the hours, you supervise, and you review and adjust. Here is the whole journey, stage by stage.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital or SNF discharge planner, a home-health/hospice partner, or a care manager (Part 10). Someone asks whether you can staff care for a specific person.

What you do:

- Take the basics (who needs care, what tasks, how many hours, where, when to start, and who pays — private, LTC insurance, VA, or regional center). Confirm the need is non-medical.

Stage 2 — Assessment & the service plan

Do an in-home assessment and build a service plan: the specific tasks (bathing, dressing, meals, housekeeping, transport, companionship), the schedule and hours, and any safety concerns. Match the plan to the client's needs and preferences.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement covering services, schedule, rate/payer, client rights, and how service can end. Assign a registered, background-cleared, TB-screened, trained aide, and make the first visit a supervised, well-introduced start.

Stage 4 — Delivering the hours

The aide delivers the scheduled care; keep visit notes and make sure the aide's skills match the client's needs. Consistency of caregiver is the single biggest driver of client satisfaction — protect the client-caregiver match.

Stage 5 — Supervision & quality

Supervise per your policy: check in on the schedule you set with the family, monitor the aide's performance and the client's changing needs, and document it. Run your quality program — satisfaction, complaints, incident reports — and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As the client's needs change, update the service plan and the hours. For VA or regional-center clients, coordinate reauthorizations with the funder. Keep authorizations current so the work you deliver is the work that gets paid.

Stage 7 — Discharge & transition

When care ends — recovery, a move to a higher level of care, or a hospice transition — follow your discharge policy, give proper notice, coordinate with the family and any funder, and document the transition. A good discharge and a follow-up call turn families into referral sources.

PART 8

How You Get Paid — Private Pay, LTC Insurance, VA & Regional Centers

In short: A California HCO is mostly a private-pay business. IHSS (the big public program) pays individual providers, not agencies. Your realistic non-private channels are VA contracts and regional-center vendorization.

Your revenue streams, in order of realism for an agency:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Your core market — start on your license; ~\$39/hr CA median (2024)
Long-term-care insurance	The client's LTC policy reimburses in-home care	A major secondary payer; bill the family or insurer
Veterans (VA)	VA Homemaker/HHA via the Community Care Network	A real agency channel — contract through TriWest (CCN Region 4)
Regional centers	Developmental-disability services (vendorized)	Bill a regional center for eligible DD clients
IHSS	County Medicaid personal care	Mostly pays INDIVIDUAL providers — agencies only via county “contract mode”

8.1 Private pay — your core market

The day your license issues, you serve private-pay clients — families paying out of pocket, typically billed by the hour with a visit minimum (often 4 hours; live-in priced separately). California homemaker care runs around \$39/hour to the client (2024 CareScout cost-of-care survey; a 2025 methodology change merged “homemaker” and “home health aide” into one “non-medical caregiver” category, so use the 2024 California figure as your benchmark and re-verify). You set your rate above your fully-loaded caregiver cost.

8.2 Long-term-care insurance

Long-term-care insurance is a significant secondary payer: many LTC policies reimburse licensed home care for personal and homemaker services once the benefit triggers are met (typically needing help with two or more ADLs, or cognitive impairment). You bill the family or the insurer and help the client with the claim — being a licensed HCO is usually required for the policy to pay.

8.3 Veterans — a real agency channel

- **VA Homemaker / Home Health Aide (H/HHA):** the VA's non-medical in-home program; aides work for an agency that holds a VA contract — the veteran doesn't hire directly. This is your entry point to serve veterans.
- **Join the Community Care Network:** California is in CCN Region 4, administered by TriWest — request a contract via TriWest's provider site or 877-226-8749 (the VA is re-competing CCN contracts through 2026, so confirm the current administrator).
- **Aid & Attendance:** an enhanced VA pension (2025–2026: up to about \$2,424/month for a single veteran, \$1,558 for a surviving spouse — verify on VA.gov) that a veteran can apply toward your private-pay care.

8.4 Regional centers & the IHSS reality

For clients with a developmental disability, you can become a vendored provider of your area's regional center and bill for in-home respite, personal assistance, and supported living — a genuine public-funding channel for that population. IHSS, by contrast, is not really an agency channel: it pays individual providers the client hires, and an HCO can only participate through county “contract mode,” which exists in a minority of counties. Don't build your plan on IHSS agency revenue — build it on private pay, LTC insurance, VA, and (where you serve the DD population) regional centers.

PART 9

What It Costs & How Long It Takes

In short: Budget the HCO license, the bond & insurance, caregiver registry/training costs, payroll float, and (optionally) accreditation. Plan to launch on private pay as soon as the license issues.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
HCO license	2-year California HCO license	\$5,603 / 2 years
Insurance & bond	\$1M/\$3M liability + workers' comp + \$10K dishonesty bond (+ auto/abuse)	Get quotes — annual
Business setup	LLC-1 \$70 + \$800 franchise tax + LLC-12 \$20, EIN free, legal	~\$900 + legal
Caregiver registry	\$35 per aide (2-yr) + Live Scan + TB, per caregiver	Per aide
Caregiver training	5-hr entry + 5-hr annual per aide (can be online)	Per aide (low)
Software	Scheduling / records / invoicing / payroll platform	Monthly SaaS
Payroll float	You pay caregivers before clients/insurers pay you	Working capital — plan for it
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission (see 9.2)	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. Home care is payroll-heavy: you pay caregivers weekly, but LTC insurers and the VA pay you weeks later. Private-pay clients (billed in advance or on short terms) smooth this out. Line up a business line of credit before you scale.

9.2 If you pursue accreditation

- **Why:** some VA contracts, long-term-care insurers, and franchise brands prefer or require accreditation, and it's the on-ramp if you later add skilled home health.
- **Who:** ACHC, CHAP, and The Joint Commission all accredit home care; expect roughly \$7,000–\$14,500 over a 3-year cycle plus staff time.
- **When:** most non-medical agencies launch on the state license and add accreditation in year two.

9.3 Timeline

Early	Entity, insurance & bond	Form the LLC; bind the \$1M/\$3M policy, workers' comp, and \$10K bond
Weeks	Policies & training plan	Write the P&P Manual, job descriptions, and caregiver training plan
Submit	HCO application	File the HCS 200 packet + \$5,603 with the Home Care Services Bureau
Parallel	Register your aides	Live Scan, TB, and 5-hr entry training so caregivers are ready
At license	Start private pay	Serve private-pay & LTC-insurance clients
Then	Add VA / regional centers	Contract with TriWest (CCN) and/or vendorize with a regional center

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$39/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.5 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay caregivers every week or two, but long-term-care insurers, the VA, and regional centers pay you weeks later. Private-pay clients (billed in advance) smooth this out — but keep several weeks of payroll in reserve or line up a business line of credit before you scale.
- **Unbillable overhead:** your own time, your scheduler/coordinator, office and phone, background checks, and the aide TB/training you fund — none of it is billable, all of it is real.
- **Turnover churn:** every caregiver who quits costs you re-recruiting, re-registering, re-Live-Scanning, and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a VA, LTC-insurance, or regional-center claim that misses an authorization or timely filing simply doesn't get paid. Reconcile every payment and chase exceptions weekly — unreconciled claims are money you earned and lost.

PART 10

Getting Your First Clients

In short: *Because the business is private pay, clients come through referral relationships — discharge planners, home-health/hospice partners, care managers, senior communities, and the aging network. Build these before you open.*

10.1 The referral sources

- **Hospital & SNF discharge planners:** the highest-volume source of “needs help at home now” referrals — introduce yourself, leave a one-pager, and make same-day starts easy.
- **Skilled home-health & hospice agencies:** natural partners — they provide the intermittent skilled care, you fill the hourly personal-care gap; build reciprocal referrals.
- **Geriatric care managers & elder-law attorneys:** Aging Life Care professionals place clients with vetted agencies; elder-law and fiduciary professionals advise families arranging care.
- **Senior living communities:** independent-living, assisted-living/RCFE, and CCRC residents often need supplemental one-on-one care — become a preferred provider.
- **The aging network — AAAs & ADRCs:** California has 33 Area Agencies on Aging and the Aging & Disability Resource Connections (find services at aging.ca.gov) that field families looking for in-home help — get listed and known.
- **2-1-1 California & regional centers:** list your services with 2-1-1, and if you vendorize, your regional center places developmental-disability clients.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the registry & caregivers

Who	Contact	What to verify / ask
CDSS — Home Care Services Bureau	(877) 424-5778 · HCSB@dss.ca.gov	The HCO application (HCS 200); the \$5,603 fee; the bond/insurance proof; unannounced visits
CDSS — Care Provider Management Branch	1-888-422-5669 · guardian@dss.ca.gov	Home Care Aide registration; Live Scan; current fingerprint fees; clearances/exemptions
Home Care Aide Registry	cald.dss.ca.gov/hcsregistry	Registering aides (\$35/2-yr); checking clearance status
Training / TB requirements	HSC §§ 1796.44–.45	The 5-hr entry + 5-hr annual training; the TB rule; the 2027 sunset
Live Scan vendor (DOJ)	oag.ca.gov (Live Scan)	Fingerprinting logistics and current DOJ/FBI fees

Business, insurance & bond

Who	Contact	What to verify / ask
California Secretary of State	916-657-5448 · bizfileOnline.sos.ca.gov	Forming your LLC (LLC-1, \$70); the LLC-12
Franchise Tax Board	ftb.ca.gov	The \$800 minimum franchise tax (no first-year waiver now)
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.4	\$1M/\$3M liability + workers' comp + \$10K bond (HSC § 1796.37); auto & abuse add-ons
NPPES / CMS	nppes.cms.hhs.gov	A Type 2 NPI (for VA/regional-center billing)

Funding & referrals

Who	Contact	What to verify / ask
VA Community Care / TriWest (Region 4)	877-226-8749 · ProviderServices@TriWest.com	Joining the CCN; the VA Homemaker/HHA program; the current TPA
Your Regional Center	dds.ca.gov/rc (find your center)	Vendorization; eligible services; the rate
County IHSS / Public Authority	your county IHSS office	Whether the county uses “contract mode” (the only IHSS agency lane)
Long-term-care insurers	the client's policy / carrier	Benefit triggers; whether a licensed HCO is required; how to bill
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; cost; payer requirements

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (33 in CA)	aging.ca.gov (find services) · C4A (c4a.info)	Local information & referral for older adults
Aging & Disability Resource Connections	aging.ca.gov (ADRC) · ADRC@aging.ca.gov	The “No Wrong Door” access point for long-term services & supports
2-1-1 California	dial 211 · 211ca.org	Free statewide referral line — list your services
Hospital / SNF discharge planners	your local hospitals & SNFs	The highest-volume private-pay referral source

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a California non-medical home care agency is very doable — but go in clear-eyed that it's a private-pay business, not a Medicaid-billing one. Take it one rung at a time: form the business and bind the required insurance and bond, get your HCO license, register and train your caregivers on the Home Care Aide Registry, launch on private pay and long-term-care insurance, add VA and regional-center clients, and build referral relationships with discharge planners and the aging network. Verify each fee and contact as you go, and lean on the people in Part 11.

***Reminder.** Prepared for PolicyReady.org and grounded in the Home Care Services Consumer Protection Act (HSC § 1796.10 et seq.) and current CDSS information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with CDSS/HCSB and qualified counsel before you rely on it. Confirm the current HCO fee, the aide training/TB rules (note the 2027 sunset), and the insurance/bond limits before you rely on them.*